



CELL THERAPY PROCESS MANUFACTURING MAPS

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Authors

Bayer Healthcare

Scott Probst
Amol Thote

bluebird bio

Steve Goodman

Bristol Myers Squibb

Randy Schweickart
Matthew Westoby

Janssen

Frederik De Vos
Devon Zimmerman

Lonza

Senthil Ramaswamy
Kesav Reddy

Merck

Kara Levine
Samantha Sbardella

Merck & Co. Inc. USA

Zhimei Du

Roche Genentech

David Shaw

Sanofi

Alexandra Grella

Sangamo Therapeutics

Nikul Patel

BioPhorum

Steven Wall

About BioPhorum

BioPhorum's mission is to create environments where the global biopharmaceutical industry can collaborate and accelerate its rate of progress, for the benefit of all.

Since its inception in 2004, BioPhorum has become the open and trusted environment where senior leaders of the biopharmaceutical industry come together to openly share and discuss the emerging trends and challenges facing their industry.

Growing from an end-user group in 2008, BioPhorum now comprises over 90 manufacturers and suppliers deploying their top 3,500 leaders and subject matter experts to work in seven focused Phorums, articulating the industry's technology roadmap, defining the supply partner practices of the future, and developing and adopting best practices in drug substance, fill finish, process development and manufacturing IT. In each of these Phorums, BioPhorum facilitators bring leaders together to create future visions, mobilize teams of experts on the opportunities, create partnerships that enable change and provide the quickest route to implementation, so that the industry shares, learns and builds the best solutions together.

1.0

Introduction

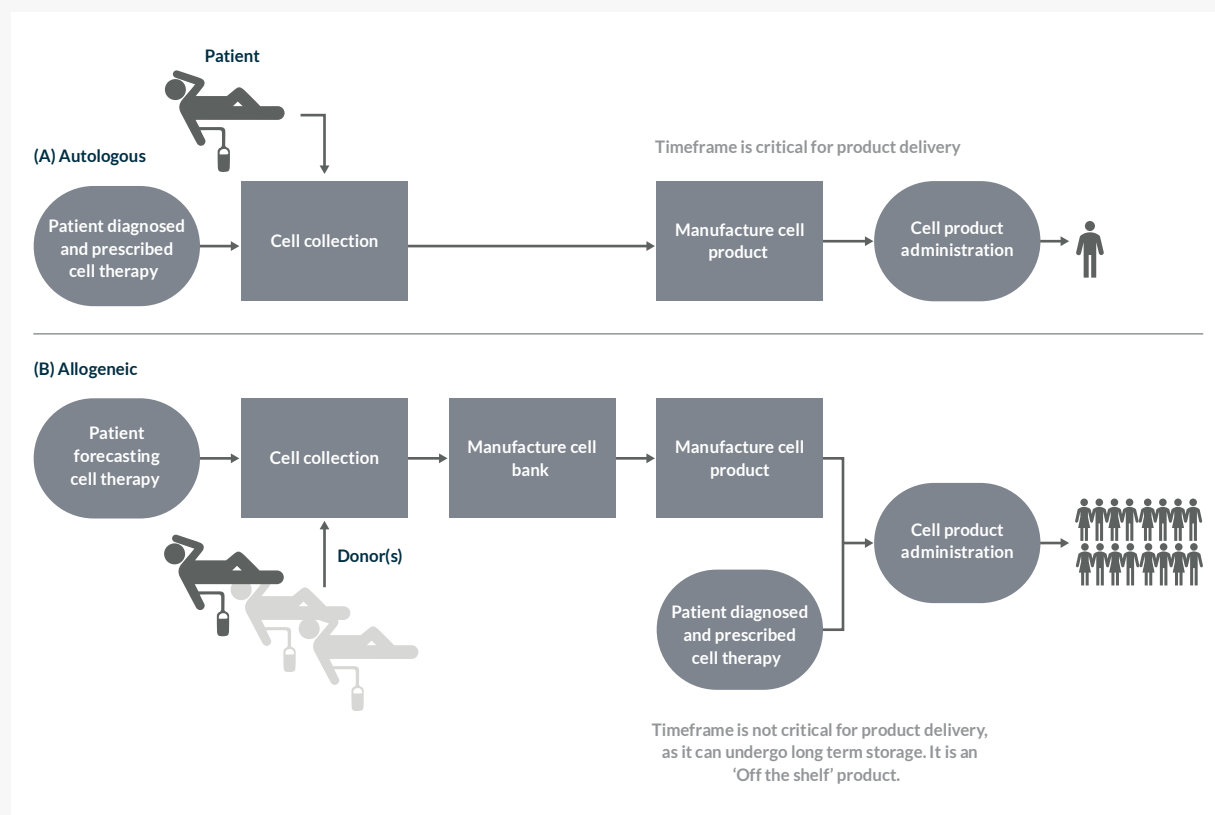
Until recently, there were limited options for patients seeking treatment for ailments or diseases relating, but not limited to, cancer, inflammatory diseases, tissue replacement and blood disorders. However, over this time, a new therapeutic pillar began to emerge through the support of academic centers. Today, the academic vision of shifting treatment paradigms has fully evolved to clinically proven results and market approval – all of this is in thanks to cell and gene-modified cell therapies.

Recent industry reports show that there are over 630 clinical trials (and growing) underway for cell and gene-modified cell therapies with nearly 55% of these in Phase II*. In addition, many companies apply for the Regenerative Medicine Advanced Therapy (RMAT) designation to speed up the development process. The numbers are staggering and these therapies pose unique manufacturing challenges. Unlike the conventional biologics industry with the ability to leverage templated manufacturing processes, cell and gene-modified cell therapy production standards have yet to be established.

Here, BioPhorum's Cell & Gene Therapy Commercialization workstream has generated a set of cell and gene-modified cell therapy process maps that represent example processes, unit operations and platform technologies being used in cell and gene therapy manufacturing today. A high-level overview of autologous and allogeneic cell therapy manufacturing is shown in Figure 1. As the focus of this article is cell therapy, manufacturing details for viral gene editing agents (such as lentivirus), are not shown. Viral vector manufacturing and other processes to support *in vivo* and *ex vivo* gene therapies are being mapped by another team in the BioPhorum Cell & Gene Therapy Commercialization workstream www.biophorum.com/download/gene-therapy-process-map/

**Reference: Alliance for Regenerative Medicine, Q3 2019 Report*

Figure 1: Autologous and allogeneic cell therapy manufacture overview



The intent of the cell therapy process maps provided in the appendices is to reflect the current state of selected manufacturing processes within the cell therapy field, form a common frame of reference for scientists and engineers working within the field, and support collaboration to enable process improvements benefitting future cell therapy patients.

2.0

In/out of scope

Due to the large and diverse nature of the cell and gene therapy field, it was agreed that detailing every process, technology and unique aspect would not be possible. Additionally, due to current variability in processes between companies, depicting every process route would result in highly complicated (and confusing) process maps. Therefore, to enable ease of use of the detailed process maps a number of aspects were agreed to be out of scope. Below is a list of aspects that the team included in and out of scope.

In scope

- example high-level workflows describing one possible route for manufacturing cell and gene-modified cell therapies for both autologous and allogeneic formats
- maps that represent methods in use by the majority of cell therapy manufacturers today
- high-level steps for workflows, including incoming ancillary materials (e.g. buffers, media, reagents, virus) and outgoing final product
- brief explanation of the process and example technologies for each process step box (not an exhaustive list)

Out of scope

- exhaustive list of all processes, methods, and technologies in use for therapy manufacturing today
- process maps describing fully manual or fully closed and automated processes for autologous and allogeneic manufacturing
- manufacturing processes for ancillary materials, e.g. buffers, reagents, viral vectors
- clinic-related steps including administration to patient, e.g. thawing at the clinic
- in-process control (IPC) / process analytical technology (PAT) testing
- quality control sampling points – study is focused on manufacturing aspects and assume each company will test at required points as per own control strategy and regulatory requirements
- discussion of specific technologies or supply chain logistics
- detailed step-by-step methodology indicating each equipment/handling step
- timing – processing or turnaround
- variation of scale
- specific sources, cell types or therapies not covered – bone marrow, mesenchymal stem cells (MSCs), embryonic stem cells, haemopoietic stem cells (HSCs)

3.0

Overview of the process maps

Each box is a hyperlink to the full process map, with legend, in the appendices.



Legends

The following is a list of legends indicating what each process map depicts. These are not intended as a full description of the process, as the maps are intended to be used independently. The text below each process map box provides a brief explanation/function and some example technologies for that step. Additionally, each map contains steps above the workflow describing points for addition of buffers, raw materials, and key ancillary materials.

Appendix 1: Autologous cell therapy, patient treatment cycle

Autologous patient treatment cycle from diagnosis through administration at the clinical site. Example includes cell preparation scenarios for local and centralized manufacturing. The block flow diagram also includes links to more detailed associated workflows covering manufacturing of viral and non-viral gene-edited products (appendix 2 and appendix 3). Color-coded pathways represent different shipping/cryopreservation options for cells.

Appendix 2: Autologous cell therapy, viral gene editing manufacturing process

Model autologous workflow for manufacturing chimeric antigen receptor T cell therapies (CAR-T) using **viral** gene editing technologies. Steps include cell prep, selection, activation, genetic modification, expansion, and harvest/formulation to generate fresh and cryopreserved CAR-T cells. Final containers and labels are included as ancillary materials due to criticality of labelling during process and potential for this step to occur in manufacturing suite.

Appendix 3: Autologous cell therapy, non-viral gene editing manufacturing process

Model autologous workflow for manufacturing chimeric antigen receptor T cell therapies (CAR-T) using **non-viral** gene editing technologies. Steps include cell preparation, selection, activation, genetic modification, expansion, and harvest/formulation to generate fresh and cryopreserved CAR-T cells. Final containers and labels are included as ancillary materials due to criticality of labelling during process and potential for this step to occur in manufacturing suite.

Appendix 4: Allogeneic cell therapy, patient treatment cycle

Allogeneic patient treatment cycle from diagnosis through administration in the clinic. Associated appendix 7 indicates separate workflow detailing methods for generation of target cells via differentiation of fresh and frozen induced pluripotent stem cells (iPSCs).

Appendix 5: Allogeneic cell therapy, iPSC generation and cell banking

Generation and banking of genetically modified iPSCs from patient samples for good manufacturing practice (GMP) manufacturing. Steps include sample collection, selection, genetic modification, cloning, cell banking and bank testing. Process-flow diagram describes production of starting materials used in appendix 6.

Appendix 6: Allogeneic cell therapy, iPSC cultivation and freezing

Large-scale expansion, harvest, and cryopreservation of iPSCs from working cell banks (WCB). Appendix 5 indicates links from separate workflow detailing generation of the iPSC WCB. Process results in starting materials for iPSC differentiation into target cell types (appendix 7).

Appendix 7: Allogeneic cell therapy, iPSC differentiation and filling

Differentiation of iPSCs into target cells from WCBs of cryopreserved cells generated in workflow appendix 6. Steps cover differentiation, selection, and formulation of fresh and cryopreserved cell products for shipment to the clinical administration site. Work precedes patient treatment cycle described in appendix 4.

Appendix 8: Allogeneic cell therapy, CAR-T cell selection, editing and expansion

Manufacturing of CAR-T cell therapy using viral gene editing. Steps include cell selection, activation, genetic modification (gene knockout, delivery), and expansion. Map describes production of starting materials for cell product generation depicted in appendix 9.

Appendix 9: Allogeneic cell therapy, CAR-T cell product

Processing of cell product using starting materials produced in workflow appendix 8. Steps include selection of CAR+ TCR- cells through harvest, formulation, and cryopreservation. Final containers and labels are included as ancillary materials due to criticality of labelling.

4.0

Conclusions

Standard methodologies for generating therapeutic cell products (with or without gene modification) continue to change as the field matures. BioPhorum has identified significant gaps in publicly available information describing industrial practices for production of cell and gene-modified cell therapies. The process maps created by BioPhorum's Cell & Gene Therapy Commercialization workstream, included in this study, describe a variety of known manufacturing processes in the field of cell and gene-modified cell therapy. They are intended to enable discussion across the field and support accelerated commercialization of novel modalities.

The Commercialization workstream will continue to leverage these example process maps as a tool to identify outstanding challenges in the field and potential mitigation strategies to drive improvement and standardization in cell and gene-modified cell therapy manufacturing.

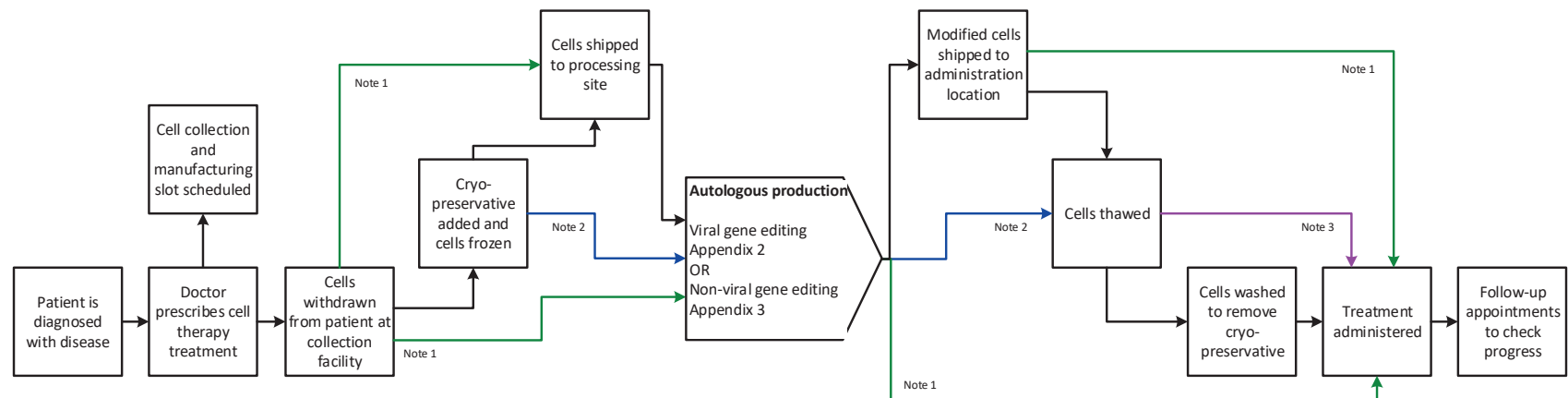
Appendices

Process maps

The following 9-page appendices contain the process maps.

These process maps are available to download and print in A3.

Appendix 1: Autologous cell therapy, patient treatment cycle



Explanation/Function

Collect apheresis or whole blood

Wash, concentrate and resuspend cells for freezing in vials or bags

Cells shipped to processing facility

Cell modification as detailed in linked referenced maps

Hospital or clinic

Thawing of cryopreserved cells

Cryopreservative replaced with final formulation solution

Single or multiple doses are administered at the hospital or clinic

Patient returns to doctor to evaluate treatment effectiveness

Technology platforms

Apheresis apparatus

Tangential flow filtration or centrifugation

Dry ice shipper
Liquid nitrogen transport dewar

Dry ice shipper
Liquid nitrogen transport dewar

Waterbath or dry thawing device

Tangential flow filtration or centrifugation

Infusion of cells in blood stream

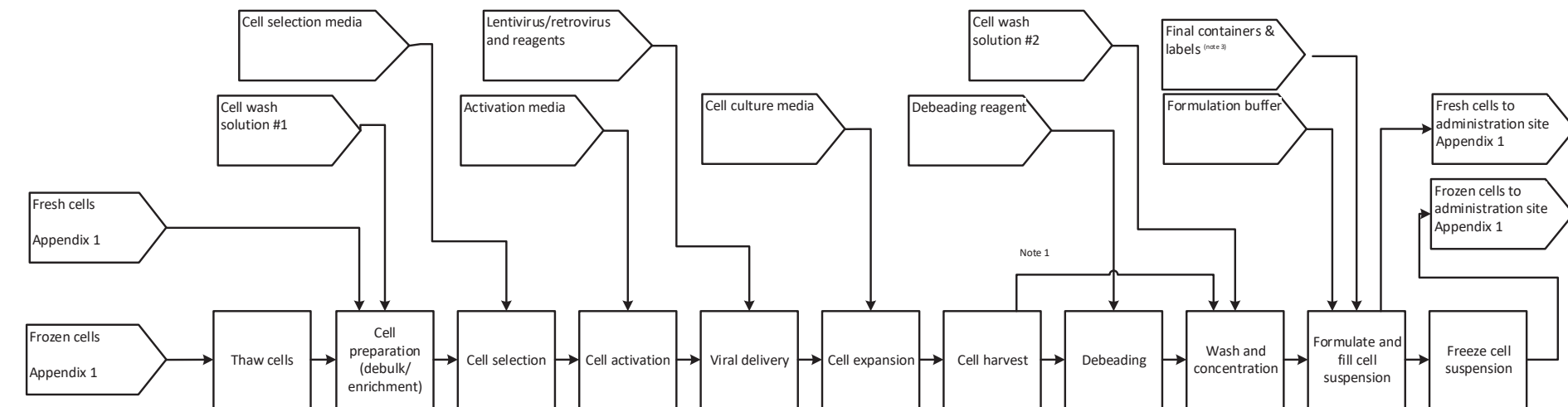
Surgical implantation of cells in target tissue

Notes

1. Represents case in which cells are not frozen. Cells processed locally or shipped without cryopreservation.
2. Represents case in which cells are processed locally and cryopreserved as part of manufacturing process.
3. Product has injectable cryopreservative.

[Back to legend text](#)

Appendix 2: Autologous cell therapy, viral gene editing manufacturing process



Explanation/Function

Thaw samples in bags/cryovials

Reduce contaminating cell types (e.g. RBCs/platelets) from apheresis or whole blood

Select CD3+, CD8+, CD4+ T-cells from PBMCs

Stimulate T-cell growth and enhance uptake of viral material

Deliver genetic material to nucleus
Washout excess virus

Expand gene modified T cells in static or agitated culture systems

Pool cell culture material and prepare to purify and formulate

Remove cell selection or activation media from cell suspension

Purify cell suspension and concentrate to the desired volume for administration

Place cell suspension in solution for administration or cryopreservation

Freeze suspension to preserve cells during shipment or storage

Technology platforms

Water bath
Dry thawing device

Density gradient centrifugations

Affinity purification using magnetic or other beads

Beads coated with activators such as CD3/CD28
Non-bead technologies
Interleukins

Lentivirus or retrovirus delivery system
Transduction using spinoculation or reagents

Single-use bioreactors
Gas permeable bags
Flasks
Cell culture media, cytokines, growth factors and/or serum

Depth filtration
Tangential flow filtration,
Centrifugation

Magnetic cell separation

Tangential flow filtration
Centrifugation

Tangential flow filtration
Centrifugation

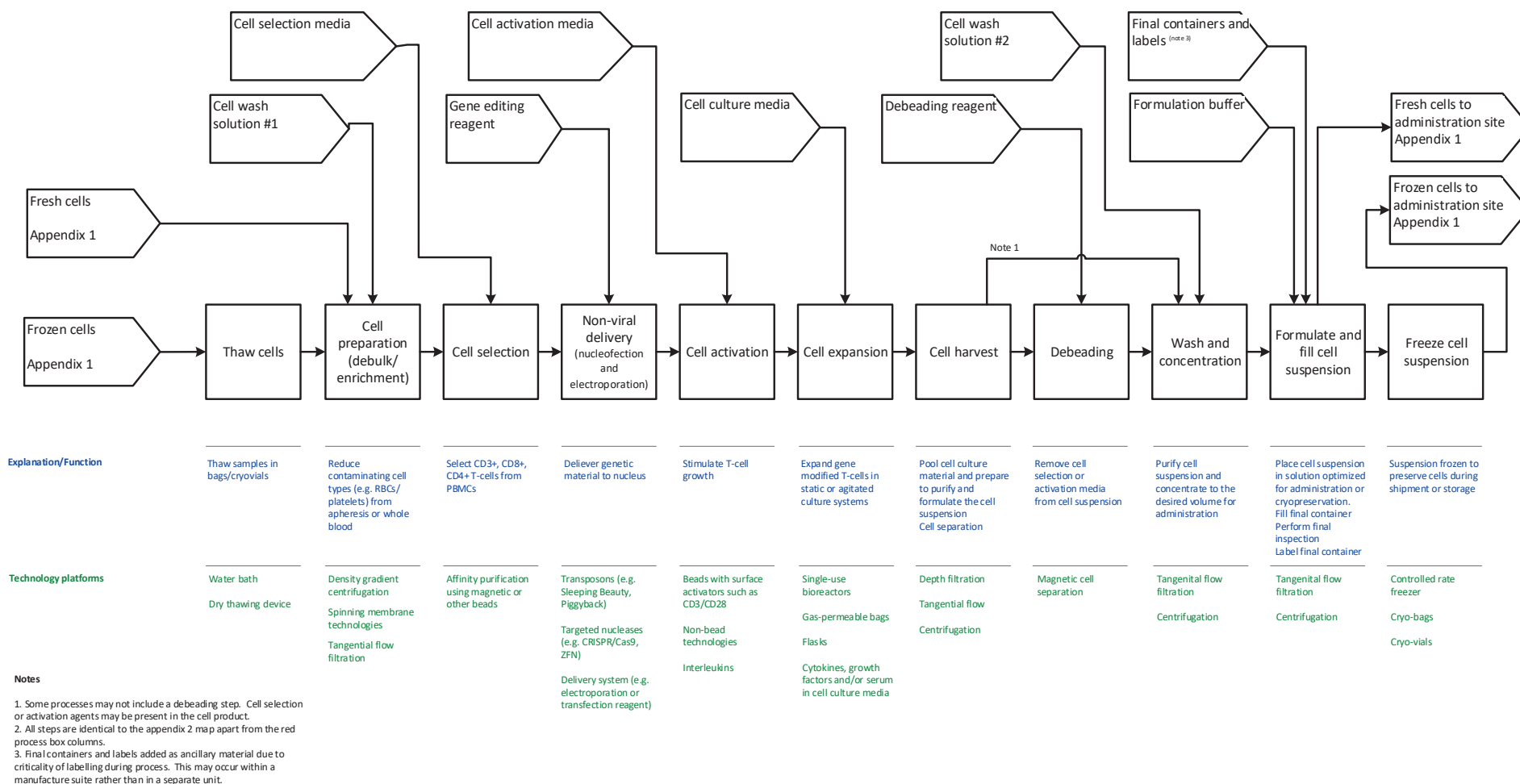
Controlled rate freezer
Cryo-bags
Cryo-vials

Notes

1. Some processes may not include a debeading step. Cell selection or activation agents may be present in the cell product.
2. All steps are identical to the appendix 3 map apart from the red process box columns.
3. Final containers and labels added as ancillary material due to criticality of labelling during process. This may occur within a manufacture suite rather than in a separate unit.

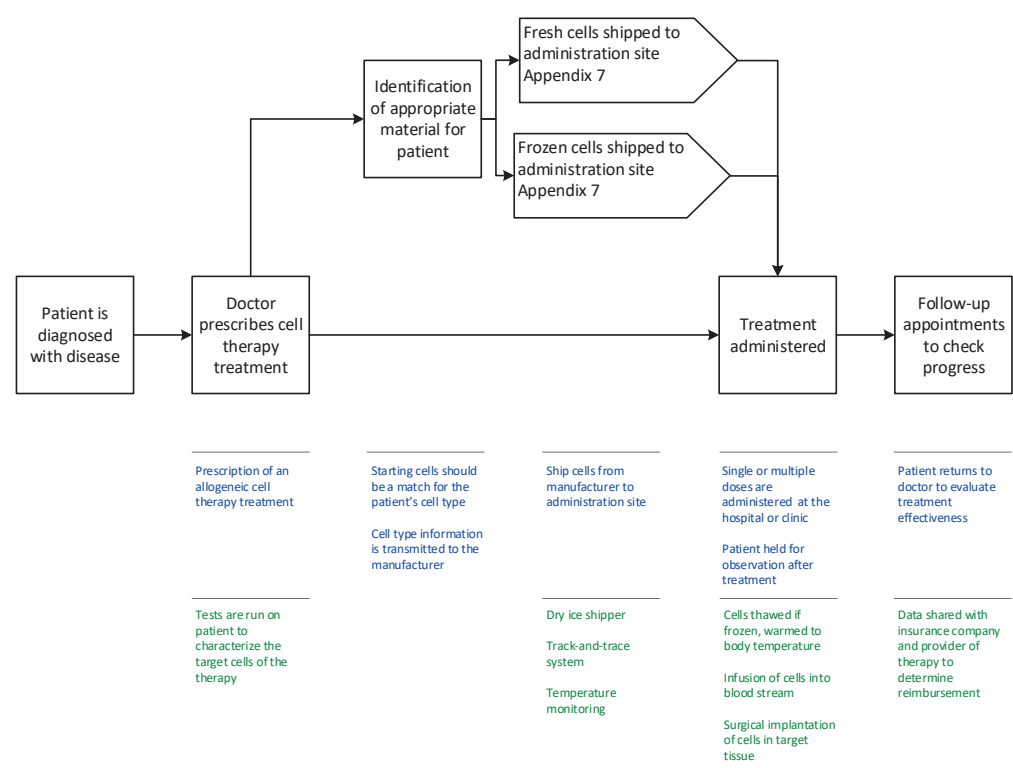
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Appendix 3: Autologous cell therapy, non-viral gene editing manufacturing process



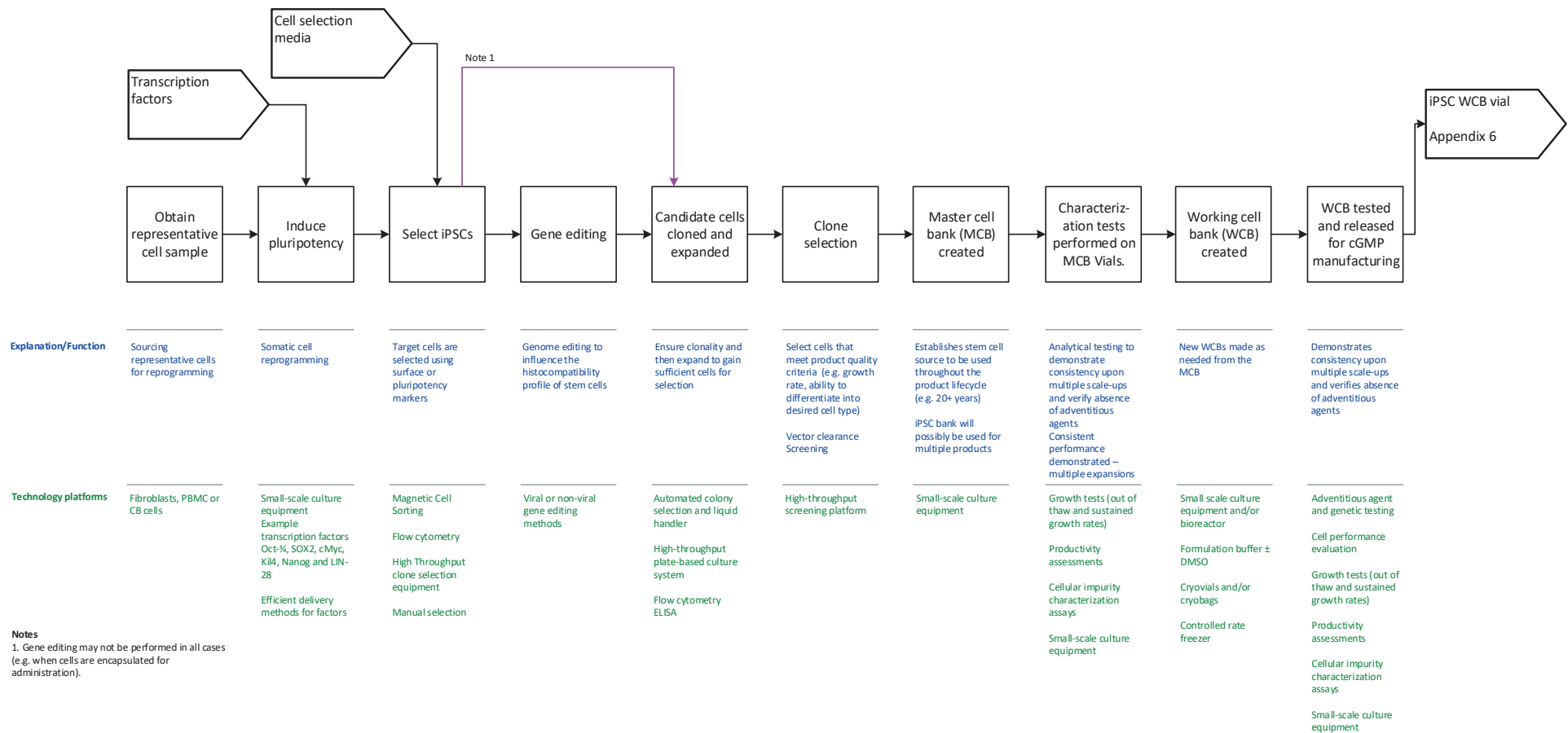
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Appendix 4: Allogeneic cell therapy, patient treatment cycle



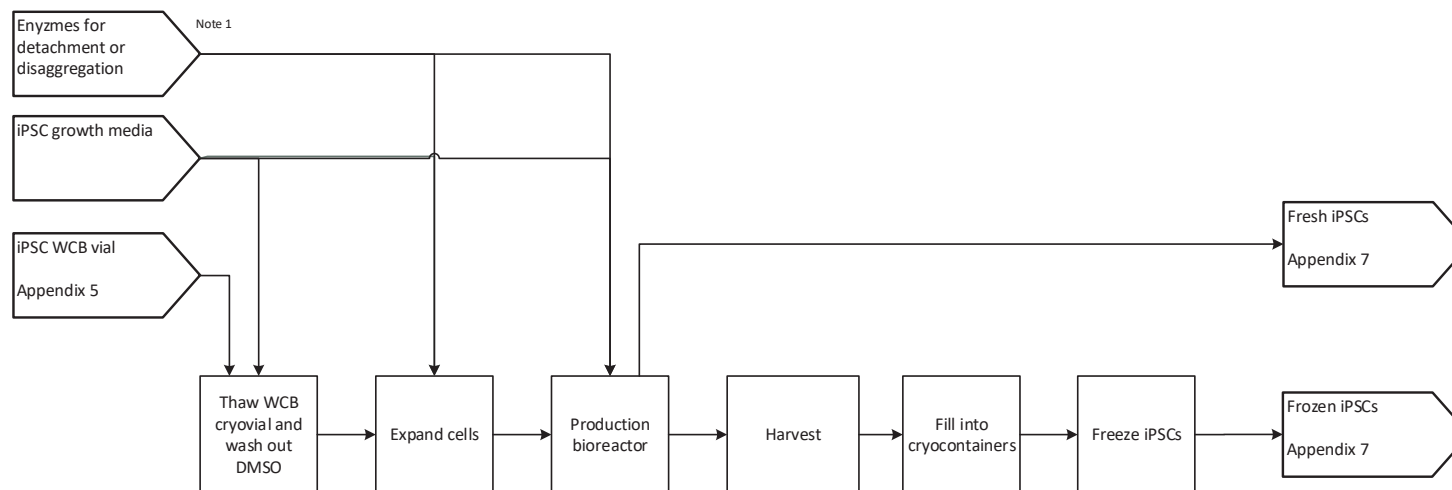
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Appendix 5: Allogeneic cell therapy, iPSC generation and cell banking



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Appendix 6: Allogeneic cell therapy, iPSC cultivation and freezing



Explanation/Function

Thaw cells, wash out DMSO and cryopreservative

Expand iPSCs in planar or suspension culture systems

Expand iPSCs to achieve target culture volume and cell density

Adherent cells detached or aggregates dissociated between passages

Wash Suspension, adjust cell concentration, and add cryopreservative

Aliquot formulated culture into containers for freezing

Freeze cells to maximize viability upon thaw

Technology platforms

Waterbath or dry-thawing device
Centrifuge

Adherent cell culture platform. Enzymatic detachment of cells at each passage (if required)
Suspension platform using rho-kinase inhibitor medium
Controlled aggregate sizes

Suspension cultures: Single-use bioreactors, media for enzyme dissociation (if required)

Adherent cultures: planar surfaces, microcarriers or hydrogels

Centrifugation

Sterile cryobag manifolds
Cryovials filled in aseptic, robotic filling machine

Manual filling using a pipette under laminar air flow

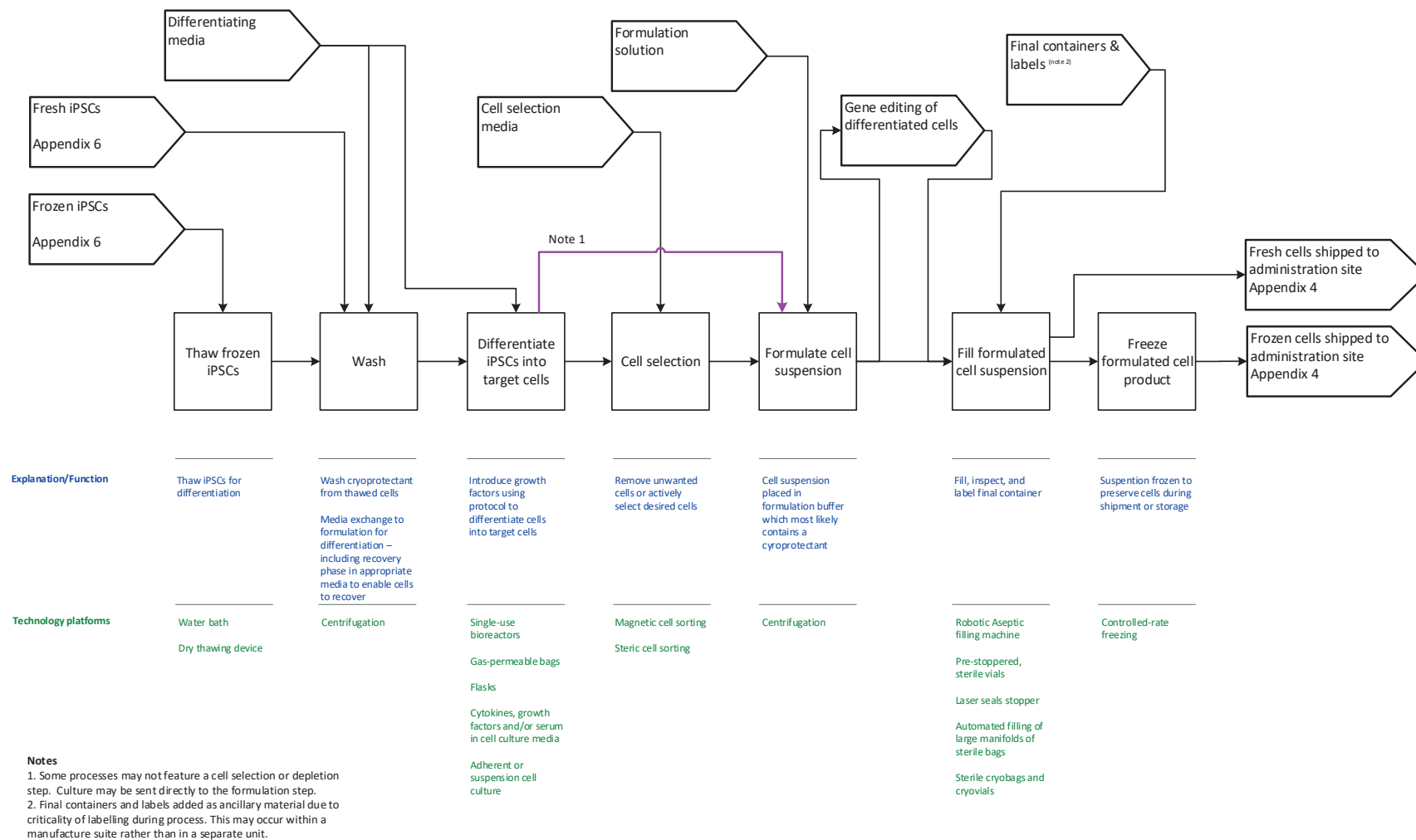
Controlled rate freezer

Notes

1. Not needed in every process.

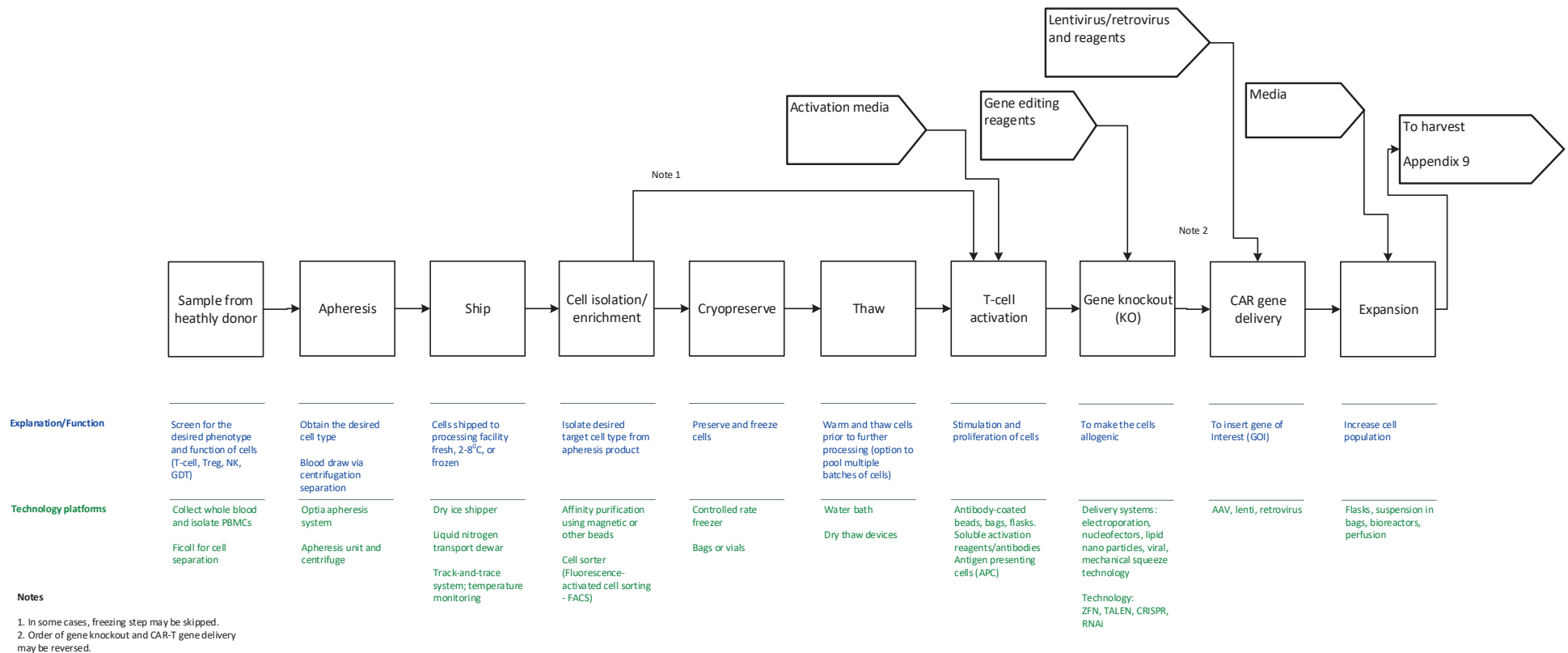
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Appendix 7: Allogeneic cell therapy, iPSC differentiation and filling



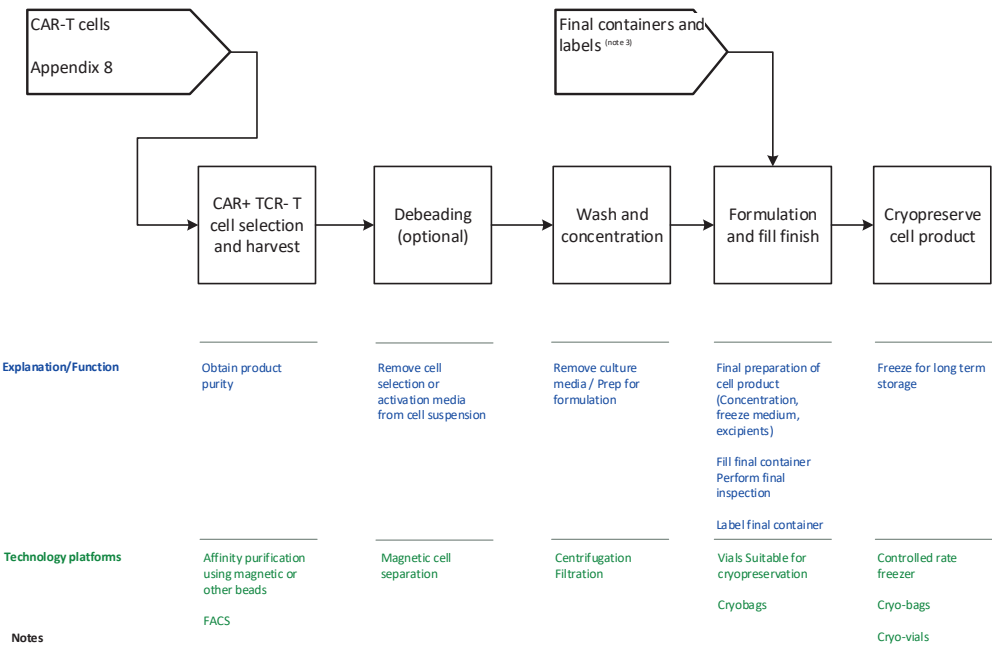
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Appendix 8: Allogeneic cell therapy, CAR-T cell selection, editing and expansion



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Appendix 9: Allogeneic cell therapy, CAR-T cell product



Notes

1. In some cases, freezing step may be skipped.

2. Order of gene knockout and CAR-T gene delivery may be reversed.

3.Final containers and labels added as ancillary material due to criticality of labelling during process. This may occur within a manufacture suite rather than in a separate unit.

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